

Opening the conversation

For the person presenting this page. These are notes to adapt, not a script to finish.

Begin here

"Pastor, before we talk about what the church needs to do next, I want to make room for what you've been carrying.

"There may be things you have walked through that never had room to settle. Sunday came again. Someone else needed you. You kept going.

"I don't want to assume I know what that has cost you. You don't have to explain it in this room. We can make room for a private conversation with someone you trust. And the church can begin sharing the weight without asking you to tell every part of your story."

Pause. Let people pass or ask to stop. Offer prayer with permission. Silence is not an invitation to press for disclosure.

The Heartbeat

Read Mark 6:30-34. The returning apostles reported back; Jesus invited them to rest. Then the crowd arrived and He responded with compassion. Preserve both: His care for the disciples and for the people they served. This is not a clinical recovery plan. Our application is that those carrying ministry need care, too.

In Gethsemane Jesus voiced His sorrow and asked trusted disciples to remain near (Matthew 26:36-46). Point to His honesty, not claim He had burnout or PTSD.

Our primary foundation remains 2 Timothy 1:6-7, read with verse 8: God's gift and power in costly ministry. Do not use the flame language to tell an exhausted Pastor to push harder or treat distress as weak faith.

Ask the church's leaders

What have we learned to call normal that keeps asking too much of the Pastor?

If the Pastor stepped away to receive care, who would actually carry the work?

What can we remove or share now, rather than wait for a crisis?

Care and follow-through

Protect choice and privacy

Offer privately: "Would it help to talk about what ministry has been costing you?" If welcomed: "What would help you feel supported? Who would you feel safe receiving help from?" People may decline. Explain confidentiality and its safety/reporting limits before sensitive conversation; never promise absolute secrecy. Do not ask for trauma, symptom, or treatment disclosures in a public or board meeting. If leaders have contributed to harm, help identify independent safe support.

A first response the church can own

Agree on one immediate relief the Pastor welcomes: remove a nonessential demand, arrange real coverage, or help connect with agreed professional care. Name who will arrange it, when it starts, and how interruptions will be handled. Confirm covering Staff/Volunteers have willingness, capacity, time, and support. Record operational changes, not private disclosures. Check whether relief actually happened; do not measure healing by productivity.

Do not send a wounded Pastor back into an unchanged fire.

Know the limits of this conversation

Burnout and PTSD are not interchangeable. Burnout is an occupational phenomenon, not a medical diagnosis. Persistent distress, disrupted sleep, intrusive memories, avoidance, or feeling on alert can warrant professional attention; they are not a diagnostic checklist. Use "trauma-related stress symptoms" only when traumatic exposure is relevant. Diagnosis and treatment belong with qualified professionals. Prayer must not replace needed care.

Rest, leave, role changes, and return decisions need individual discernment and relevant professional guidance. Do not promise quick recovery or let staffing needs set a healing deadline. The dimming metaphor refers to human capacity, not God becoming absent.

If urgent help is needed

In the United States, call or text 988 for suicide risk or an emotional crisis. For immediate danger call 911. Stop the advisory discussion and help connect with urgent support.

Before the next chapter, make room for actual care and relief. No one needs to finish this book to receive help.

Sources: Mark 6:30-34 (NKJV); Matthew 26:36-46 and 2 Timothy 1:6-8 (NIV). WHO, "Burn-out an occupational phenomenon"; VA National Center for PTSD, "PTSD Basics"; 988 Lifeline, "What to Expect." Online sources checked September 16, 2026.
Draft for review. Qualified clinical review is still needed before church use.